

STROKE: ROUTINE CARE

Sudden onset of one or more of the following suggests a stroke (or a transient ischaemic attack (TIA) if symptoms lasted < 24 hours and resolved completely):

- Weakness or numbness of the face, arm or leg, especially on one side of the body
- Difficulty speaking or understanding
- Blurred or decreased vision in one/both eyes or double vision
- Difficulty walking, dizziness, loss of balance or co-ordination

Give urgent attention to the patient with a new stroke/TIA:

- If oxygen saturation < 94% or respiratory rate $\geq$ 30, give face mask oxygen.
- Keep patient nil by mouth until swallowing is formally assessed.
- Check glucose: if < 3 (< 4 if diabetes) $\rightarrow$ 17.
- Avoid treating BP $\geq$ 140/90 as this may worsen stroke. If BP $\geq$ 220/120, discuss with specialist about need for pre-referral treatment.
- Decide where to refer the patient depending on when symptoms started:
 - If patient can reach hospital within 3 hours of onset of symptoms, *refer urgently* for thrombolysis (to specialist stroke unit if available).
 - If patient cannot reach hospital within 3 hours of onset of symptoms, refer same day and give single dose **aspirin** 300mg (avoid if on long-term anticoagulant or headache/neck stiffness) if fully conscious and can swallow.

Assess the patient with stroke/TIA

Assess	When to assess	Note
Symptoms	Every visit	Ask about symptoms of another stroke/TIA. Also ask about chest pain $\rightarrow$ 37 or leg pain $\rightarrow$ 65.
Depression	Every visit	In the past month, has patient: 1) felt down, depressed, hopeless or 2) felt little interest or pleasure in doing things? If yes to either $\rightarrow$ 143.
Rehabilitation needs	Every visit	Refer to appropriate therapist: physiotherapy for mobility, physiotherapy/occupational therapy for self care, speech therapist for swallowing, coughing after eating, speaking and drooling.
Palliative care	Every visit	If any of: severely disabled, worsening problems with speech or swallowing or breathing problem, also give palliative care $\rightarrow$ 170.
BP	Every visit	If known hypertension $\rightarrow$ 133. If not, check BP: if $\geq$ 140/90 $\rightarrow$ 132. If new hypertension, start treatment only 48 hours after a stroke $\rightarrow$ 133.
Diabetes risk	At diagnosis and yearly	If known diabetes $\rightarrow$ 130. If not known with diabetes, check glucose $\rightarrow$ 17.
Fasting cholesterol and triglycerides	At diagnosis if not already done	If cholesterol > 7.5 or triglycerides > 10, check TSH and refer to doctor.
HIV	At diagnosis, especially if age < 50 years	Test for HIV $\rightarrow$ 110. If HIV, give routine care $\rightarrow$ 111.
ECG	At diagnosis if not already done	If abnormal, discuss/refer.

Advise the patient with stroke/TIA

- Educate the patient that stroke/TIA is a brain attack. Quick treatment of a minor stroke or TIA can reduce the risk of a major stroke. Refer to available helpline/s $\rightarrow$ 178.
- Help patient to manage CVD risk $\rightarrow$ 129. If < 55 years (man) or < 65 years (woman), advise the first degree relatives (parents, siblings, children) to have CVD risk assessment $\rightarrow$ 127.
- Avoid oral contraceptives containing oestrogen. Advise other method such as copper IUD, injectable, progestogen-only pill $\rightarrow$ 154.

Treat the patient with stroke/TIA

- Give **aspirin** 150mg daily for life. Avoid if < 30 years, haemorrhagic stroke, previous peptic ulcer, dyspepsia or on anticoagulant. If prosthetic heart valve, valvular heart disease or atrial fibrillation, refer for warfarin instead.
- Give **simvastatin**¹ 40mg² at night for life, regardless of cholesterol if patient had an ischaemic stroke. If patient develops muscle pain/cramps, reduce dose to 10mg at night. Avoid if pregnant or liver disease.
- Check that patient is up to date with COVID-19 vaccine. If age > 65 years, or known HIV or heart or lung disease, give **influenza vaccine** 0.5mL IM yearly.

¹If HIV positive on lopinavir/ritonavir or atazanavir/ritonavir, avoid simvastatin, give instead **atorvastatin** 10mg at night. ²If on amlodipine, reduce simvastatin dose to 20mg at night. No dose adjustment needed for rosuvastatin, pravastatin, atorvastatin.